

Outpatient Empiric Antimicrobial Treatment Recommendations - Pediatric Gold Card

DIAGNOSIS	PATHOGENS	DRUGS OF CHOICE ¹	ALTERNATE DRUGS ¹	COMMENTS
1. Skin & Skin Structure Infections				
1.1. Uncomplicated bites (dog, cat, human) Prophylaxis for higher-risk bites, or treatment for established infection High risk may include cat bites, deeper puncture bites, or bites to hand/foot/face/genital area. Refer to AAP Redbook.	Viridans Streptococci <i>P. multocida</i> Anaerobes <i>Staph. aureus</i> (MSSA and MRSA) <i>E. corrodens</i>	AMOXICILLIN/CLAVULANATE 40-45 mg/kg/day PO ("standard-dose," see table for dosing)	PCN allergy, or failure on AMOX/CLAV, or suspected MRSA (e.g. patient or family history): CLINDAMYCIN ² 30 mg/kg/day PO divided TID (max: 450 mg/dose) -plus either- ≥2 months: TMP/SMX 10 mg/kg/day of TMP PO divided BID (max: TMP 160 mg/dose) -or- 3rd GENERATION CEPHALOSPORIN* PO <i>*CEFDINIR or CEFPODOXIME³, see table for dosing and info (including use in setting of PCN allergies).</i>	- Dura days prop days treat - Cons tetan prop
1.2. Non-purulent cellulitis	Group A Streptococci <i>Staph. aureus</i> (including MRSA)	CEPHALEXIN 50-75 mg/kg/day PO divided TID (max: 500 mg/dose)	Severe PCN allergy (e.g. anaphylaxis), or allergy to/failure of CEPHALEXIN, or suspected MRSA (e.g. patient or family history): CLINDAMYCIN ² 30 mg/kg/day PO divided TID (max: 450 mg/dose) -or- ≥2 months: TMP/SMX 10 mg/kg/day of TMP PO divided BID (max: TMP 160 mg/dose) -or- DOXYCYCLINE 4 mg/kg/day PO divided BID (max: 100 mg/dose)	- Dura days - For e cellul need antib hosp MRS, or lo durat - TMP, the d choic A stre provi cover viable mon skin a struc infec

1.2.1. Purulent cellulitis or superficial abscess	<i>Staph. aureus</i> (including MRSA) <i>Group A Streptococci</i>	≥2 months: TMP/SMX 10 mg/kg/day of TMP PO divided BID (max: TMP 160 mg/dose)	CLINDAMYCIN ² 30 mg/kg/day PO divided TID (max: 450 mg/dose) -or- DOXYCYCLINE 4 mg/kg/day PO divided BID (max: 100 mg/dose)	• Duration: 5-7 days; follow up if drainage persists; appropriate wound care • Abscesses may be drained if fluctuant
1.3. Impetigo	<i>Staph. aureus</i> (including MRSA) Group A Streptococci	Few lesions: MUPIROCIN 2% ointment topically BID Many lesions or large lesions: CEPHALEXIN 50 mg/kg/day PO divided TID (max: 500 mg/dose)	Severe PCN allergy (e.g. anaphylaxis), or allergy to/failure of CEPHALEXIN, or suspected MRSA (e.g. patient or family history) Many or large lesions: CLINDAMYCIN ² 30 mg/kg/day PO divided TID (max: 300 mg/dose) -or- ≥2 months: TMP/SMX 10 mg/kg/day of TMP PO divided BID (max: TMP 160 mg/dose)	• Duration: 7-10 days
2. Head and Neck Infections, Superficial				
2.1. Conjunctivitis (age ≥30 days)	<i>Staph. aureus</i> <i>Strep. spp.</i> <i>H. influenzae</i>	TRIMETHOPRIM SULFATE + POLYMYXIN B ophthalmic drops TID	MOXIFLOXACIN 0.5% ophthalmic drops TID -or- OFLOXACIN 0.3% ophthalmic drops TID -or- ERYTHROMYCIN 0.5% (5 mg/g) ophthalmic ointment TID	• Duration: 7-10 days • Many cases are viral and resolve with supportive care; warm compresses BID. • These are for purulent infection; conjunctivitis without purulence is NON-infectious and does not require treatment.

2.2. Lymphadenitis (bacterial)	<i>Staph. aureus</i> (including MRSA) Group A Streptococci <i>Anaerobes</i> (occasionally)	CEPHALEXIN 50-75 mg/kg/day PO divided TID (max: 1 g/dose)	Associated periodontal disease or poor dental hygiene: AMOXICILLIN/CLAVULANATE* 40-45 mg/kg/day ("standard-dose," see table for dosing) Severe PCN allergy (e.g. anaphylaxis), or allergy to/failure of cephalexin, or suspected MRSA (e.g. patient or family history): CLINDAMYCIN² 30 mg/kg/day PO divided TID (max: 450 mg/dose) -or- ≥2 months: TMP/SMX 10 mg/kg/day of TMP PO divided BID (max: TMP 160 mg/dose)	• Duration: days; resolution of signs and symptoms • Abscesses: may be difficult to culture • Acute lymphadenitis often tends to be inflammatory, bilateral, nontender lymph nodes; less likely to respond to antibiotics; may require additional workup; appearance of risk factors; consider etiology; react to mycobacteria; onco • Consider <i>Bartonella henselae</i>; serologic testing; cat exposure; see 2 • Consider discussion with person; not necessary; any risk; infection; a primary concern

2.2.1. Bartonella lymphadenitis (Cat Scratch Disease):	<i>Bartonella henselae</i>	Symptomatic care	Large or painful adenitis: AZITHROMYCIN 10 mg/kg/dose PO on day 1 (max: 500 mg/dose), then 5 mg/kg/dose PO daily on days 2-5 (max: 250 mg/dose)	- Unco <i>Barto</i> - aden - limiti - spon 2 to 4 - Per A - Acad - Pedia - most - thera - antib - haste - resol
2.3. Orolabial herpes infections in immunocompetent children (non-genital, non-ocular)				
2.3.1. Primary/initial infection (e.g., gingivostomatitis) in children >3 months	Herpes simplex virus	ACYCLOVIR 80 mg/kg/day PO divided QID (max: 800 mg/dose)	VALACYCLOVIR (≥ 3 months old): 40 mg/kg/day PO divided BID (max: 1 g/dose)	- Dura - Initia - symp - treata - initia - than - after - not b - VALA - suspe - be co - from - twice - but c - to pr - be m - ACYC

2.3.2. Recurrent herpes infection (e.g. cold sores) in children >6 months	Herpes simplex virus	Episodic treatment: ACYCLOVIR 80 mg/kg/day PO divided QID (max: 800 mg/dose) x 7 days	Episodic treatment (≥ 12 years old): VALACYCLOVIR 2g PO BID x 1 day	Initial symptomatic treatment initiated rather than after not b
		Daily suppressive therapy: ACYCLOVIR 40 mg/kg/day divided BID (max: 400 mg/dose)	Daily suppressive therapy: (≥ 12 years old): VALACYCLOVIR 500 mg* PO once daily <i>*In adolescents with frequent recurrences (i.e., ≥ 10 per year), doses of 1,000 mg/day may be more effective. Monitor for nephrotoxicity in patients with underlying renal issues.</i>	- Durability of treatment (Reevaluation of annual) - Increased frequency of episodic treatment of br - Consider suppressive therapy in children with frequent recurrences occur times despite therapy address triggers exposure Consider need
2.4. Otitis externa	<i>Staph. aureus</i> (including MRSA) <i>Pseudomonas aeruginosa</i>	OFLOXACIN 0.3% otic solution 5 drops BID	NEOMYCIN/ POLYMYXIN B/ DEXAMETHASONE susp 3 to 4 drops TID	- Durability (external need symptomatic resolution) - Consider performance of medical body - If persistent suspect patient tubes OFLO

2.5. Preseptal/periorbital cellulitis	<i>Staph. aureus</i> (including MRSA) Group A Streptococci <i>Strep. pneumoniae</i> <i>H. influenzae</i> Other Gram-negatives & anaerobes (with sinus source)	Suspected skin source: CEPHALEXIN 50-75 mg/kg/day divided TID (max: 500 mg/dose)	Suspected skin source: Severe PCN allergy (e.g. anaphylaxis), or allergy to/failure of CEPHALEXIN, or suspected MRSA (e.g. patient or family history): ≥2 months: TMP/SMX 10 mg/kg/day of TMP PO divided BID (max: TMP 160 mg/dose) -or- CLINDAMYCIN ² 30 mg/kg/day PO divided TID (max: 450 mg/dose)	• Duration: 5-7 days, on ex infection response to therapy • Differentiate between preseptal and orbital cellulitis. Examine orbit, include ophthalmology if pain, move to appropriate setting for cellulitis if external high delay of orbital or co-sinus. Manage orbit, usual admittance, imaging, specific consult, IV antibiotics. • Mild swelling, signs of underlying infection, sinusitis.
		Suspected sinus source: AMOXICILLIN/CLAVULANATE ES-600 susp 90 mg/kg/day PO divided BID ("high-dose," see table below for dosing)	Suspected sinus source: PCN allergy: 2nd/3rd GENERATION CEPHALOSPORIN* PO <i>*CEFDINIR or CEFPODOXIME³. May use CEFUROXIME tabs for older children. See table for dosing and info (including use in setting of PCN allergies).</i> PCN allergy, or allergy to/failure of AMOX/CLAV, or suspected MRSA (e.g. patient or family history): CLINDAMYCIN ² 30 mg/kg/day PO divided TID (max: 450 mg/dose)	
2.6. Tinea capitis	<i>Trichophyton tonsurans</i> <i>Microsporum canis</i>	GRISOFULVIN microsize 20 mg/kg/day PO daily x 6 weeks	TERBINAFINE ⁴ PO daily x 4-6 weeks: • 10- <20 kg: 62.5 mg/day, • 20-40 kg: 125 mg/day • >40 kg: 250 mg/day -or- FLUCONAZOLE 6 mg/kg/day PO daily x 3- 4 weeks	• Addition of SELENE, SULFONAMIDES, KETOCONAZOLE, shampoos, likely to recur. • TERBINAFINE available as 250 mg tabs, FLUCONAZOLE available as suspension, tabs.
3. Head and Neck Infections, Respiratory Tract				

3.1. Dental infections	Aerobes & anaerobes: Strep. spp. <i>Bacteroides</i> <i>Fusobacterium</i> <i>Actinomyces</i> <i>Prevotella</i>	AMOXICILLIN/CLAVULANATE 40-45 mg/kg/day PO ("standard-dose," see table for dosing)	PCN allergy: CLINDAMYCIN ² 30 mg/kg/day PO divided TID (max: 450 mg/dose)	• Duration • Definition should determine dental
3.2. Otitis media	<i>Strep. pneumoniae</i> <i>H. influenzae</i> <i>Moraxella catarrhalis</i>	AMOXICILLIN:80-90 mg/kg/day PO divided BID (max: 1g/dose)	Second-line therapy (failure of treatment after 48-72 hours, or recurrence): AMOXICILLIN/CLAVULANATE ES-600 susp 90 mg/kg/day PO divided BID ("high-dose," see table for dosing) PCN allergy: 2nd/3rd GENERATION CEPHALOSPORIN* PO Third-line therapy: CEFTRIAXONE 50 mg/kg/dose IM daily x 3 days (max: 1g/dose) *CEFDINIR or CEFPODOXIME ³ . May use CEFUROXIME tabs for older children. See table for dosing and info (including use in setting of PCN allergies).	• Treatment duration (excluding CEFT <2 yrs 2-5 yrs ≥6 yrs days) • Antibiotic not in otitis effusion (OME) • Suggest discussion and patient CEFT • Acute in young is unlikely may careful and t consi
3.3. Pharyngitis (streptococcal)	Group A Streptococci (GAS)	AMOXICILLIN 50 mg/kg/day PO daily (max: 1 g/dose) x 10 days -or- PENICILLIN VK: • (≤27 kg): 250 mg/dose PO BID x 10 days • (>27 kg): 500 mg/dose PO BID x 10 days -or- BENZATHINE PENICILLIN G: • (<27 kg): 600,000 units/dose IM x 1 dose • (≥27 kg): 1.2 million units/dose IM x 1 dose	Non-severe PCN allergy: CEPHALEXIN 40 mg/kg/day PO divided BID (max: 500 mg/dose) x 10 days Severe PCN allergy (e.g. anaphylaxis): CLINDAMYCIN ² 30 mg/kg/day PO divided TID (max: 300 mg/dose) x 10 days -or- AZITHROMYCIN 12 mg/kg/dose PO daily (max: 500 mg/dose) x 5 days	• Obtain testin • Optic amox to 25 500 r twice

3.4. Sinusitis, acute bacterial	<i>Strep. pneumoniae</i> <i>H. influenzae</i> <i>Moraxella catarrhalis</i>	Non-severe symptoms meeting either “persistent and non-improving” or “worsening after initial improvement” criteria[†]: Amoxicillin 90 mg/kg/day PO divided BID (usual max: 1g/dose)	Non-severe symptoms and PCN allergy: 2 nd /3 rd GENERATION CEPHALOSPORIN* PO * <i>CEFDINIR or CEFPODOXIME³. May use CEFUROXIME tabs for older children. See table for dosing and info (including use in setting of PCN allergies).</i> -or- CLINDAMYCIN ² 30 mg/kg/day PO divided TID (max: 450 mg/dose)	• Duration: severe 5-7 d symp treat 7-10 • [†]Diag criteria: bacter include one c follow • Severe symp 4 day ≥39° purul disch facial
		Severe symptoms[†], or non-severe symptoms with no improvement after 3 days of amoxicillin: Amoxicillin/clavulanate ES-600 susp 90 mg/kg/day PO divided BID (“high-dose,” see table for dosing)	Severe symptoms, and either PCN allergy or no improvement after 3 days of AMOX/CLAV: LEVOFLOXACIN • 6 months-4 years: 16-20 mg/kg/day PO divided BID • ≥5 years: 10 mg/kg/dose PO daily (max: 500 mg/dose) -or- CLINDAMYCIN ² 30 mg/kg/day PO divided TID (max: 450 mg/dose) -plus- 2 nd /3 rd GENERATION CEPHALOSPORIN* PO * <i>CEFDINIR or CEFPODOXIME³. May use CEFUROXIME tabs for older children. See table for dosing and info (including use in setting of PCN allergies).</i>	• Non-persi non-symp least (e.g., disch cough) • Non-wors symp initial impr (e.g., or na disch) • Saline may in ge stero FLUT preve bacter in ch allerg • Avoid deco and antih • Chro may to an may treati discu HNS.

4. Pulmonary Infections				
4.1. Community Acquired Pneumonia, 4 months – 4 years	Respiratory viruses <i>Strep. pneumoniae</i> <i>H. influenzae</i> <i>Moraxella catarrhalis</i>	AMOXICILLIN 90 mg/kg/day PO divided BID	Second-line therapy: AMOXICILLIN/CLAVULANATE ES-600 susp 90 mg/kg/day PO divided BID ("high-dose," see table for dosing) PCN allergy: CLINDAMYCIN ² 30 mg/kg/day PO divided TID -or- ≥6 months: LEVOFLOXACIN 16-20mg/kg/day PO divided BID	• Duration of therapy depends on severity of illness and response to treatment. For CAP, treatment should last at least 10-14 days. • Viral pneumonia is usually self-limiting and does not require antibiotics. • Consider corticosteroids for severe CAP or for persistent symptoms and evidence of prolonged symptoms. • Presence of complications may require longer treatment. • *Mycoplasma pneumoniae (atypical pneumonia) is uncommon in children. It is associated with prolonged illness, malaise, throat pain, fever, and development of a persistent cough 3-5 weeks after onset. Crackles are typically absent on exam.
4.2. Community Acquired Pneumonia, ≥5 years	<i>Strep. pneumoniae</i> <i>M. pneumoniae</i> <i>Staph. aureus</i>	AMOXICILLIN 90 mg/kg/day PO divided BID (max: 1 g/dose) If clinical and radiographic findings are characteristic of atypical pneumonia*, use AZITHROMYCIN alone: AZITHROMYCIN 10 mg/kg/dose (max: 500 mg/dose) PO on day 1, then 5 mg/kg/dose (max: 250 mg/dose) PO daily on days 2-5	PCN allergy: CLINDAMYCIN ² 30 mg/kg/day PO divided TID (max: 450 mg/dose) -or- LEVOFLOXACIN 8-10 mg/kg/dose PO daily (max: 750 mg/dose)	• *Mycoplasma pneumoniae (atypical pneumonia) is uncommon in children. It is associated with prolonged illness, malaise, throat pain, fever, and development of a persistent cough 3-5 weeks after onset. Crackles are typically absent on exam.
5. Genitourinary Tract Infections				

5.1. Cystitis (afebrile UTI)	<i>E. coli</i> <i>Proteus</i> spp. <i>Klebsiella</i> spp. <i>Enterococcus</i> spp.	CEPHALEXIN 50 mg/kg/day PO divided TID (max: 500 mg/dose) x 5- 7 days -or- NITROFURANTOIN: ≥1 month-12 years (macrocrystals; generic Macrochantin): 5-7 mg/kg/day PO divided QID x 5-7 days ≥12 years old (dual release; generic Macrobid): 200 mg/day PO divided BID x 5 days	Severe PCN allergy (e.g. anaphylaxis): NITROFURANTOIN: ≥ 1 month- < 12 years (macrocrystals; generic Macrochantin): 5-7 mg/kg/day PO divided QID x 5-7 days ≥ 12 years old (dual release; generic Macrobid): 200 mg/day PO divided BID x 5 days -or- ≥2 months: TMP/SMX 10 mg/kg/day of TMP PO divided BID x 3-5 days (3-day duration is for mild disease patients ≥ 12 years) -or- 3rd GENERATION CEPHALOSPORIN* PO x 5-7 days * CEFIXIME, CEFPODOXIME ³ , or CEFDINIR. See table for dosing and info (including use in setting of PCN allergies).	• Adjust treatment on u suscept • Consider consi highl organ resist sever ceph • Diagn be m with urine cathe supra is nee • Do n or tre asym bacte • Cons exter antib for sl impr symp youn
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5.2. Pyelonephritis and febrile UTI	<i>E. coli</i> <i>Proteus spp.</i> <i>Klebsiella pneumoniae</i>	CEPHALEXIN 75 mg/kg/day PO divided TID (max: 1g/dose)	Severe PCN allergy (e.g. anaphylaxis): 3rd GENERATION CEPHALOSPORIN* PO * CEFIXIME, CEFPODOXIME3, or CEFDINIR. See table for dosing and info (including use in setting of PCN allergies) -or- CIPROFLOXACIN 30 mg/kg/day PO divided BID (max: 500 mg/dose) If initially unable to tolerate PO: Consider a one-time dose of CEFTRIAXONE 50-75 mg/kg IM (max: 1 g/dose) along with a PO course and close follow-up	• Duration: 7-10 days. Improvement should be seen within 48h of starting therapy. If no improvement by day 3, consider alternative therapy. • Consider admission if severe symptoms, dehydration, or if unable to tolerate oral therapy. • Adjust dosing based on renal function and susceptibility. Consider CIPROFLOXACIN for high-risk infections (Pseudomonas, other resistant organisms).
5.3. Sexually Transmitted Diseases, adolescents • Anyone with a positive STD test should prompt testing for other STDs, including HIV. • Treat or advise treatment of sexual partners.				

5.3.1 Pelvic Inflammatory Disease	Mixed infection: <i>Neisseria gonorrhoeae</i> <i>Chlamydia trachomatis</i> Vaginal flora Anaerobes	CEFTRIAXONE: 45-149 kg: 500 mg/dose IM x 1 ≥150 kg: 1g/dose IM x 1 -plus- DOXYCYCLINE 100 mg/dose PO BID x 14 days -plus- METRONIDAZOLE 500 mg/dose PO BID x 14 days	Pregnancy or CEFTRIAXONE allergy: Outpatient alternatives are limited. Consult with ID. CEFTRIAXONE is not cross-reactive with PCN (safe to give for most PCN allergies).	- Cons: parenteral therapy improves 72 hr - DOXY not recommended pregr
5.3.2. Chlamydia (Urethritis/Cervicitis, Rectal or Pharyngeal Chlamydia) (consider testing multiple sites)	<i>Chlamydia trachomatis</i>	No adherence concerns: DOXYCYCLINE 100 mg/dose PO BID x 7 days (Preferred)	No adherence concerns: AZITHROMYCIN 1 g/dose PO x 1 [DOT*] -or- LEVOFLOXACIN 500 mg/dose PO daily x 7 days	- Retest: reinfection, patie mont - Addi: cure, recor for: - Preg: pati - Rec: chla, trea, azitl - You: with, abu - Pers: sym, con, rein, adh, con *DOT: obse, drug, admi, clinic, for al, dose - Cons: swab, recta, MSM
		Adherence concerns: AZITHROMYCIN 1 g/dose PO x 1 [DOT*] <i>If rectal chlamydia is diagnosed, doxycycline is preferred therapy unless pregnant.</i>		
		Pregnant: AZITHROMYCIN 1 g/dose PO x 1 [DOT*]	Pregnant: AMOXICILLIN 500 mg/dose PO TID x 7 days	

5.3.3. Gonorrhea (Urethritis/Cervicitis, Rectal, and Pharyngeal Gonorrhea) (consider testing multiple sites)	<i>Neisseria gonorrhoeae</i>	First-line (including if PCN- allergic): CEFTRIAXONE: 45-149 kg: 500 mg/dose IM x 1 ≥150 kg: 1g/dose IM x 1 -and- If chlamydial infection is not excluded, treat chlamydia concurrently as above in 5.3.2.	CEFTRIAXONE allergy: AZITHROMYCIN 2 g/dose PO x 1 [DOT*] -and- GENTAMICIN 240 mg/dose IM x 1 If IM not feasible, or partner therapy: CEFIXIME 800 mg/dose PO x 1 [DOT*] -and- If chlamydial infection is not excluded, treat chlamydia concurrently as above in 5.3.2.	- Test c reco only i phary of gc week treati for re all pa mont - ID co for pi do ne to an - GENT not reco pregn *DOT obse drug admi clini for al dose
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¹ Maximum dose is equal to the standard adult dose
² Suspension may be unpalatable to young children
³ Oral suspension may be Non-Formulary
⁴ Oral capsules are Non-Formulary

SUMMARY OF DOSING RECOMMENDATIONS FOR ORAL ANTI-
INFECTIVES

Antibiotic	Usual Dosing Recommendations (All doses oral unless indicated otherwise)	Maximum Recommended Dose	Comments
Amoxicillin (<i>Amoxil</i>)	Standard-dose: 45mg/kg/day divided BID or TID (depending on indication) High-dose: 90 mg/kg/day divided BID	Absolute max: 4 g/day Usual max: 2 g/day	

Amoxicillin/clavulanate (AMOX/CLAV, Augmentin, generic)	(Dosage based on amoxicillin component)	(Based on amoxicillin component [^])	*For patients ≥40kg, 875mg-125mg (7:1) tabs are KP formulary and usually sufficient for "high dose" indications. For severe infections with "high-dose" indications, 1000mg BID of amoxicillin component can be achieved using 7:1 suspension or 16:1 tablets (AMOX/CLAV 1000mg-62.5mg extended-release tablets are Non-Formulary).
"Standard-dose" formulations (ratio): "AMOX/CLAV 125" suspension: 125mg-31.25mg/5 mL (4:1) "AMOX/CLAV 250" suspension: 250mg-62.5mg/5 mL (4:1) "AMOX/CLAV 500" tab: 500mg-125mg (4:1) "AMOX/CLAV 200" suspension: 200mg-28.5mg/5 mL (7:1) "AMOX/CLAV 400" suspension: 400mg-57mg/5 mL (7:1) "AMOX/CLAV 400" chewable tab: 400mg-57mg/5 mL (7:1) "AMOX/CLAV 875" tab: 875mg-125mg (7:1) "High-dose" formulations (ratio): "AMOX/CLAV 600 ES" suspension: 600mg-42.9mg/5ml (14:1) "AMOX/CLAV 875" tab: 875mg-125mg (7:1)* "AMOX/CLAV 1000 XR" extended-release tablets: 1000mg-62.5mg (16:1)* (Non-Formulary)	<3 months (any infection type): 30 mg/kg/day divided BID (<i>using 4:1 formulation</i>) ≥3 months, "standard-dose" (e.g. bite wounds, cellulitis, dental infections, lymphadenitis, urinary tract infection): 40 mg/kg/day divided TID (<i>using 4:1 formulation</i>) -or- 45 mg/kg/day divided BID (<i>using 7:1 formulation</i>) ≥3 months, "high-dose" (AOM, sinusitis, pneumonia, preseptal/periorbital cellulitis): 90 mg/kg/day divided BID <40kg: use 14:1 suspension ≥40kg: use 7:1 tablets or suspension OR if for sinusitis or pneumonia use 16:1 extended-release tablets (Non-Formulary)*	4:1 formulations = 500mg/dose TID (1500 mg/day) 7:1 formulations = 875mg/dose BID (1750 mg/day) 14:1 or 16:1* formulations = usual max: 2g/day; absolute max: 4 g/day	^Max amoxicillin dose differs by formulation to prevent excessive clavulanate (cause of diarrhea).
Penicillin VK (<i>Veetids</i>)	<12 years: 25-50 mg/kg/day divided TID/QID ≥12 yrs: 125-500 mg/dose TID/QID	3 g/day	
First Generation Cephalosporin			
Cephalexin (<i>Keflex</i>)	25-100 mg/kg/day divided TID/QID	Absolute max: 1g/dose, 4 g/day Standard max: 250-500mg/dose	
Second Generation Cephalosporin (Note: Second/third-generation oral cephalosporin susceptibilities cannot be inferred from ceftriaxone susceptibility [for gram-negative organisms])			

Cefuroxime (Ceftin)	>3 months to 12 years (suspension): 20-30 mg/kg/day divided BID ≥ 13 years (tablets): 250-500 mg BID	1 g/day	- Cefuroxime can be used in the setting of PCN allergy (including anaphylaxis), as it shares no side chains with PCN is not cross-reactive. Risk of cross-reactivity in severe PCN-delayed reactions (such as Stevens-Johnson Syndrome) is not well understood. - Suspension may be unpalatable to young children. - Oral suspension may be Non-Formulary.
Third Generation Cephalosporins <i>(Note: Second/third-generation oral cephalosporin susceptibilities cannot be inferred from ceftriaxone susceptibility [for gram-negative organisms]. Third-generation oral cephalosporin can be used in the setting of PCN allergy [including anaphylaxis], as it shares no side chains with PCN is not cross-reactive. Risk of cross-reactivity in severe PCN-delayed reactions [such as Stevens-Johnson Syndrome] is not well understood.)</i>			
Cefdinir (Omnicef, generic)	>6 months <12 years: 14 mg/kg/day daily or divided BID ≥ 12 years: 600 mg/day daily or divided BID	600 mg/day	Oral capsules are Non-Formulary.
Cefpodoxime ³ (Vantin)	>2 months-<12 years: 10 mg/kg/day divided BID ≥ 12 years: 200-800 mg/day divided BID	>2 months-<12 years: 400 mg/day ≥ 12 years: 800 mg/day	- Suspension may be unpalatable to young children. - Oral suspension may be Non-Formulary
Cefixime (Suprax)	≥6 months-12 years (≤50 kg): 8 mg/kg/day daily or divided BID >50 kg or >12 years: 400 mg/day daily or divided BID	400 mg/day	Has superior Gram-negative activity compared to other 3rd gen. cephalosporins (appropriate for GU infections) but should not be used for <i>Strep. pneumoniae</i> infections (HNS infections).
Other Antimicrobial Classes			

Azithromycin (<i>Zithromax</i>)	<i>Community acquired pneumonia:</i> 10 mg/kg for 1 day (max: 500 mg), then 5 mg/kg/day (max: 250 mg) for 4 days <i>Streptococcal pharyngitis:</i> 12 mg/kg/day (max: 500 mg) once daily for 5 days <i>Sexually transmitted diseases:</i> See above urethritis/cervicitis indications	See maximum doses to left	
Ciprofloxacin (<i>Cipro</i>)	20-40 mg/kg/day divided BID	1.5 g/day	Space by 2 hours from bivalent mineral supplements (iron, calcium, magnesium, etc) or dairy.
Clindamycin (<i>Cleocin</i>)	10-40 mg/kg/day divided TID	Usual max: 300-450 mg/dose Absolute max: 600 mg/dose, 1.8 g/day	Suspension may be unpalatable to young children.
Doxycycline (<i>Vibramycin</i>)	2- 4.4 mg/kg/day daily or divided BID	200 mg/day	• Bivalent mineral supplements (iron, calcium, magnesium, etc) or dairy may impair absorption; space by 2 hours. • Causes photosensitivity; recommend sun protection. • Risk for pill esophagitis; take pill with water at least 30 min before lying flat.
Levofloxacin (<i>Levaquin</i>)	6 months <5 years years: 16-20 mg/kg/day divided BID 5-17 years: 8-10 mg/kg/dose daily	<i>Community acquired pneumonia:</i> 750 mg/day <i>Other infections:</i> 500 mg/day	Space by 2 hours from bivalent mineral supplements (iron, calcium, magnesium, etc) or dairy.
Nitrofurantoin (<i>Macrochantin, Macrobid</i>)	≥ 1 month to < 12 years old (macrocrystals; <i>Macrochantin</i>): 5 to 7 mg/kg/day PO divided QID ≥ 12 years old (dual release; generic <i>Macrobid</i>): 100mg/dose PO BID		NITROFURANTOIN is contraindicated in febrile UTIs, infants <1 month old, and if CrCl <30 mL/min.
Trimethoprim/sulfamethoxazole (TMP/SMX, co-trimoxazole [<i>Bactrim, Septra</i>])	≥ 2 months: 8-12 mg trimethoprim/kg/day divided BID DS tablet = 160-mg TMP/800-mg SMX SS tablet = 80-mg TMP/400-mg SMX Suspension = 40-mg TMP/200-mg SMX/5 mL	160 mg trimethoprim/dose	

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This document was prepared collaboratively with TPMG and SCPMG Regional Pediatric Infectious Diseases Committees. It contains suggested antibiotic therapies for common diagnoses. It is intended as a helpful reference and is not a replacement for good clinical judgment.
